The Journal of Medicine and Philosophy: A Forum for Bioethics and Philosophy of Medicine,
2025, **50**, 262–284
https://doi.org/10.1093/jmp/jhaf003
Advance access publication 7 April 2025
Original Research

The Morality of Assisted Dying

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This essay analyzes the morality of assisted dying. To do this, it is necessary to recognize that assisted dying is the outworking of a larger process. This process unavoidably begins with the key moral conception of human dignity. Emphasis upon individualism in society has caused a restructuring of the dignity concept, changing what is most highly valued. This altered concept of dignity gives rise to assisted dying, yet is morally flawed. This is because it is an understanding of dignity that minimizes people's vulnerability, dismisses coercive forces that are brought into effect, encourages undue confidence in safeguards, and removes any sustained basis for respect of the individual. Autonomy, as the primary justification for assisted dying in contemporary society, is an abbreviated understanding of the restructured concept of dignity and therefore subject to the same criticisms. Assisted dying arises from a deficient and self-defeating foundation that mark it as immoral.

KEYWORDS: *assisted dying, autonomy, dignity, euthanasia, morality*

You are not alone in the world.
Mary Graham

…critics have not failed to point out that the limits of the private and public domains are difficult to demarcate; that anything a man does could, in principle, frustrate others; that no man is an island; that the social and the individual aspects of human beings often cannot, in practice, be disentangled.
Isaiah Berlin

…in the concept of human dignity questions about the role of humanity, the rights of individuals, and the structure of our normative social and political orders are interconnected.
Marcus Düwell

Concepts, just like individuals, have their history….
Søren Kierkegaard

…histories of ethics establish, on the one hand, the dependence of moral life on traditions of enquiry, on the other hand, the progress of relativism and irrationalism under the corrupting influence of individualism.
Émile Perreau-Saussine

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I. INTRODUCTION

The emergence of assisted dying within society is indicative of a shift in understanding with profound consequences. It is presumptive and naïve to attribute the occurrence of assisted dying within society to the outworking of "progress" (Holm, 2015). One should not presume that the occurrence of assisted dying is progress in the sense of moral improvement. We should set such simplistic accounts aside. This essay seeks to view assisted dying in perspective and analyze its morality. This moral analysis is broad and *not only* subject to the norms internal to medical practice. I set forth a contextualized, embedded concept of assisted dying from which its morality is analyzed.

Assisted dying is an inclusive term comprising *physician-assisted suicide* and *voluntary active euthanasia* (Jackson, 2012). These differ as to the agent generating the final act (Bozzaro, 2018). In physician-assisted suicide, it is the patient who is the final agent who administers and ingests a lethal substance (Bozzaro, 2018). In voluntary active euthanasia, the lethal substance is administered by someone apart from the patient (Bozzaro, 2018; Bollen, Hempton, Bhatia et al., 2023). These variations are not the focus of this essay. It is their common principle that is under consideration. They all involve a person who is considered to have the capacity to judge, who demonstrates an intent to die, and who has passed a negative judgment on living (Bozzaro, 2018). Other terms similarly capture these central elements and appear in various jurisdictions and the literature. They include *voluntary assisted dying* (Onwuteaka-Philipsen, Willmott, and White, 2019), *physician-assisted dying* (Fontalis, Prousali, and Kulkarni, 2018), *medical assistance in dying* (Downar et al., 2020) and even *doctor-assisted suicide* (Ahdar, 2016). All these terms at a certain level are "joined at the hip" (Keown, 2012). Assisted dying is the umbrella term that captures their commonality and that is used in this essay. When I use the term assisted dying in this generic way, I mean the intentional and assisted ending of a person's life at that person's request.

In distinction, *non-voluntary* and *involuntary euthanasia* are carried out when a patient does not have the capacity or does not make a request to die respectively (Keown, 2012). *Passive euthanasia*—death from the withdrawal of life-prolonging medical support—is also excluded (Jackson, 2012). James Rachels (1975, 1981) highlighted the difficulty in distinguishing the moral difference between active and passive euthanasia (i.e., between "killing" and "letting die," respectively). A number of responses arguing for these differences have been given (Asscher, 2008; Stumpf and Rogalski, 2021). Reasons include moral differences in obligations, causal responsibility, intent, and even the factual composition in what may otherwise *seem* to be identical cases (Asscher, 2008; Mohindra, 2009; Stumpf and Rogalski, 2021). I consider these distinctions persuasive and take active interventions to be distinct from passive ones for the purposes of this essay. Therefore, this essay focuses on the morality of assisted dying as the result of an *active*, "killing" intervention (Rachels, 1975).

Morality is defined here as the universal behavioral code that is normative, and so directs actions as right or wrong (Gert and Gert, 2020). Morality denotes what ought and what ought not to be and is seen as independently and objectively real—the perspective of a moral realist (Shafer-Landau, 2003). Morality is not presented here as something descriptive and only relative to a particular society or culture. After being purged from customs and desires, the judgments that morality directs are rationally available and apply to all persons, not merely prescribing or proscribing relative to culture, time, history, or preference.

This essay examines the morality of assisted dying. However, there is a danger when we consider assisted dying that we analyze this idea in a state of isolation and disengage it from its reality. I argue that we need to see assisted dying as being set in a process, a set of ideas that necessarily begins with what it is to be human and that this is linked to the key moral consideration of human dignity understood in a particular way. I argue that the ascendency of individualism has resulted in a reorientation of this concept. An altered conception of dignity not only gives rise to assisted dying, but also generates forces that influence people and reinforce its usage. It is after recognizing assisted dying as embedded within this expanding process that its morality is analyzed. The altered concept of dignity that gives rise to assisted dying is found to be a deficient and self-defeating point of departure. The main justification for assisted dying perceived in contemporary society is autonomy. Autonomy is an attenuated, abbreviated expression of the altered concept of dignity and so shares the same failings. Assisted dying, whether understood in its correctly situated process—as arising from the outworking of an

altered dignity—or simply as autonomy, is based on a deficient and self-defeating foundation. These characteristics require us to see assisted dying and its moral foundation as wrong.

II. DIGNITY

The Concept of Dignity

A clue to the concept lying beneath assisted dying is to listen to the pervasive discourse. Human dignity is appealed to by both those opposing and those supporting assisted dying (Holm, 2016). "Death with dignity" is the rationale provided for assisted dying on the one hand, while the inherent "dignity" of persons is the justification for prohibiting it on the other (Cholbi, 2017). Implicit in the first is an argument in favor of self-determination, while the second holds to the inviolable nature of the person (Jacobson, 2007). Yet, despite this display of dignity's divergent use, proponents and opponents alike correctly identify dignity as the locus of morality informing assisted dying. From its position and use, it is clear that the concept of human dignity expresses a core attribute reflecting the indispensable nature of that which is valued. By examining the concept of dignity and weaving together why it is so often dismissed yet necessarily persists, its position and structure become apparent.

Here, I assert as some others have done, that dignity is a meaningful and complex concept (Hayry, 2004; Jacobson, 2007; Pullman, 2010; Foster, 2011; Duwell, 2017; Hofmann, 2020; Arnason, 2021). Dignity in its complete form is the moral core upon which we as human beings operate when it comes to issues at the edges of life. Assisted dying manifests as the result of a foundationally altered understanding of the concept of dignity.

It is therefore necessary to explore and delineate the concept of dignity, describing its status and characteristics before identifying how a metamorphosis in the dignity concept grounds assisted dying.

Dignity's Structure

The descriptions of human dignity are varied and seem to elude the consistent particularization and systematization so instinctively sought. An inflexible search to understand dignity (Schuklenk, 2010), primarily driven by a search for a concise reduction rather than for a rich, layered concept, is bound to result in frustration. An openness is required. For, without it, the dynamics of dignity and resultant influence on assisted dying legislation is not perceived.

Rather than contemplating a simplistic notion then, dignity is best conceived of as a multifaceted descriptor. The various meanings explicated may provide the appearance of disorganization and ambiguity. Nordenfelt (2004), for instance, provides four interpretations of dignity, while Schroeder (2010) and Hayry (2004) each provide five, utilizing a spread of nuanced terminology. All these interpretations fall, however, within a concept in which two overarching dimensions may be discerned. The two dimensions of human dignity include a collective, universal dimension designated as *basic dignity* and an individual dimension, designated as *personal dignity* (Pullman, 1996, 2010). The conceptual subdivision is recognized and described by others, albeit using alternative language (Jacobson, 2007). Pullman's (1996, 2010) lucid insight and terminology of *basic dignity* and *personal dignity* will suffice. It is a categorization that captures the properties of possible human moral importance comprehensively—as the individual and collective beings we are. Here, I draw on and extend this account. The numerous varieties of meanings and interpretations of dignity found in the literature remain as facets and are indeed necessary and valuable shades of meaning. They are all subsumed within the dimensions of basic and personal dignity.

The dimension of *basic dignity* is an understanding of worth, a value universally existing in all human beings. On this view, it is therefore inviolable. Basic dignity cannot be altered, removed or lost, and, similarly, cannot be achieved or earned. The German word *Menschenwürde* captures the obligation and equality of treatment because of this inherent worth (Nordenfelt, 2004). It is not contingent on human circumstance, volition, or action (Pullman, 2010). Both the saint and the villain possess this value (Schroeder, 2010). When the preamble to the Universal Declaration of Human Rights (UDHR) refers to the "…recognition of the inherent dignity and of the equal and inalienable rights of all members of the human family…," it instantiates this dimension of human dignity (UN General Assembly, 1948). Dignity precedes law and is not merely a result of it (Pullman, 2010). It is a dignity

existing in human beings as a collective and exists plainly, not admitting of different magnitudes. Basic dignity is naïve to race, class, gender, and behavior—remaining perfectly inept at learning these. This is its great strength. In this elementary and principal sense, it is the *basic* domain of human dignity.

When basic dignity is ignored or contravened, we intuit that something is amiss and that something foundational to morality is lacking. When violated in the public domain, the consensus says otherwise and reinforces its priority. Giubilini and Minerva (2013), for example, reasoned that killing a newborn should be permissible for a variety of reasons such as parental psychological or economic circumstances. They were met with vigorous widespread, and heated opposition (Biegler, 2013; Mori, 2013). The response to the suggestion of infanticide significantly revealed an affront to a core and pervasively held societal belief—one so foundational that it disturbed the sensibilities and caused public distress when offended (Biegler, 2013). It was the basic dignity dimension—something intrinsic to human beings per se—that was implicitly recognized and reacted to (Mori, 2013).

Personal dignity, by contrast, signifies a value particular to the individual in relation to others. Personal dignity is a respect and regard given to others as they both exist and strive to determine their life's course. It speaks of the individual within the community who has life goals, projects, and understandings of what they conceive of as good (Pullman, 2010). It is dependent on circumstance and may be earned, attributed, negated, threatened, or violated (Jacobson, 2007). Personal dignity may then fluctuate in response to the vicissitudes of life (Pullman, 2002).

It should come as no surprise that human dignity is correctly understood as a complex of meanings, yet with broadly discernible dimensions: one relating to a common value independent of choice, the other contingent on the individual and their perspective. Dignity is a composite term comprising the basic and personal dimensions. The one dimension of dignity is a *collective* value belonging to human beings as a group; the other dimension is an *individual*, personal one. *Both these dimensions are intrinsic to us as human beings, and neither can be dismissed*. This reflects human beings as the indivisible beings we are, both as individuals yet also as part of a collective. Together, they characterize a value that performs the work required to accord with morality. This composite concept with discernible dimensions provides the normative ground when facing issues as diverse as genocides, consent, organ trafficking and treatment of a dead human body.

The two ineradicable dimensions of dignity have long related to each other in a sustained and stable manner in Western society. While both the personal and basic dimensions are ineradicable and contribute to the dignity of a person, it is the basic dimension that has been historically understood to be dominant (Pullman, 1996). The original understanding of human dignity, weighted by the priority of basic dignity, is widely considered to have a theological origin (Nordenfelt, 2004; Foster, 2011). Its persistence in Western society is a remnant of this legacy (Nordenfelt, 2004). Its justification lies in the *Imago Dei*—the conferred standing consequent upon being made in the image of God (Hayry, 2004; Nordenfelt, 2004; Jacobson, 2007; Schroeder, 2010). The *Imago Dei* is a belief within the Christian paradigm that admits of an open system—a coexistent realm in addition to the natural. Christianity has substantiated the primacy of basic dignity- shaping the form of the dignity concept. Whether one upholds or disregards such a belief system is irrelevant. Here, I am neither endorsing or opposing this belief system and its influence on the conception of dignity per se. Rather, I am stating that it is widely held that this belief system has informed and grounded the structure of dignity in the West (Pullman, 1996; Holland, 2019). The agnostic historian and novelist Tom Holland (2019) maintains that to live in a Western country is to live in a society utterly saturated by Christian concepts and assumptions. Figure 1 summarizes the concept of dignity comprising its dimensions and their priority.

Dignity as Foundation

Dignity's position as a moral foundation or even one of importance has been dismissed by some (Schopenhauer, 1903; Harris, 1998; Macklin, 2003; Pinker, 2008; Cochrane, 2010; Holm, 2016).

a. Personal dignity dimension
Basic dignity dimension

Fig. 1. The concept of dignity (a) is depicted with its personal and basic dimensions. The dominant priority of the basic dignity dimension is indicated in bold.

There are those who bear disdain, hostility, or a sense of exasperation (Holm, 2016). With this general spirit, Macklin (2003) attempts to reduce dignity's meaning to respect for persons or their autonomy. Harris describes appeals to human dignity "… as being universally attractive yet comprehensively vague" (1998, 163). Others have resorted to the less sophisticated yet more provocative description of the "stupidity of dignity" (Pinker, 2008, 28). The concise accusation is that dignity is redundant and reactionary and fails at being determinate (Cochrane, 2010). Dignity is said to be redundant because it provides nothing of irreducible meaning and may be restated in simpler terms. The accusation of being reactionary is leveled at its use as a moral trump card to stall all sorts of progress. Dignity is said to be indeterminate in that it is used in opposing senses and therefore supposedly lacks clarity. According to some, it should therefore be purged from ethical discourse (Macklin, 2003; Cochrane, 2010; Schuklenk, 2010). Yet, for all the polemic regarding dignity as useless and a concept to be disregarded, none is new (Schopenhauer, 1903).

The grounds for dismissing dignity as a bioethical foundation are curiously infrequently *analytically* challenged (Foster, 2011). Rather, the dismissal usually takes one of two directions. First, it is rejected as being redundant, reactionary, and indeterminate (as noted above), after perhaps an initial half-hearted acknowledgement of its well-meaning intent. Second, it may be dismissed by association because it is deemed "incurably theological" (Foster, 2011, 21).

Both modes of dismissal rely on assumptions. The first assumes that the varied and opposing ways it has been used are proof of its inadequacy. However, the lack of convergence of meaning does not prove that the concept is wrong (Bagaric and Allan, 2006). Rather, the concept may be underdeveloped or inadequately conceptualized or perhaps necessarily complex. The criticism of dignity as a foundational concept may be valid while conceptual exploration and clarification may be necessary. However, this requirement does not invalidate the concept. The criticism of its nature may simply be overly demanding and unreasonable.

The second dismissal—of dignity being "incurably theological" (Foster, 2011, 21)—is too hasty and exists as a reflex cognitive dismissal with adherents rigidly sticking with the dogma that religious talk is taboo in the field (Goss and Bishop, 2021). This is of course widely propagated and believed (Schuklenk, 2018). The second dismissal based on its seeming theological affiliation is really the weakest of positions. It is a dismissal that undermines a truth or its recognition because of prejudice. Dignity (understood as inviolable nature) may still be held as foundational without appeal to the *Imago Dei*. Foster (2011) sets forth such a position substantiated by anthropological evidence and reasoning based on human thriving and not on religion. This aside, the point is that such prejudiced dismissal speaks more of the thinker and perhaps more of the academic culture from which that person has emerged (Goss and Bishop, 2021; Richards, 2022).

Dignity has been an organizing elementary unit implicit in the Western conception of morality, whether overtly recognized or not. There are good reasons that substantiate the foundational position of dignity: first, it remains as an apparent immovable and relevant concept, holding its position despite attempts at being undermined by the most ardent of dissenters (Schuklenk, 2010). That is, it demonstrates striking *persistence*; second, it remains *indispensable* in the most difficult ethical cases, providing a moral reason where other grounding concepts fail to do so. These are outlined below.

Persistence

Dignity's persistence bears witness to its irrepressible necessity. Despite the accusations that have been leveled against it, dignity's resilience testifies to its enduring and foundational nature. Dignity persists as a central presence in a multitude of influential documents, constitutions, and declarations (Rothhaar, 2010). These include international documents such as the UDHR and the constitutions of at least 162 countries (Shulztiner and Carmi, 2014). The pivotal inclusion within new or revised constitutions is now almost universal (Shulztiner and Carmi, 2014). The function of dignity in these documents, as Shulztiner and Carmi (2014) tellingly describe, is to provide a moral justification for the *entire* constitution. It is clearly often seen as an a priori truth. In this capacity, belief in dignity is not an instruction, but rather acts as a basis for the further requirements laid out in a large number of constitutions. Constitutions, of course, comprise documents of law; however, it seems reasonable to hold that they are informed and constructed on a moral core that a society esteems.

The UDHR is an ideological document grounded in human dignity and born as a reaction to the brutalities and utter disrespect for human life witnessed in the Second World War (WWII) (Shulztiner and Carmi, 2014). In view of this, it is no coincidence that Italy, Japan, and West Germany (the defeated coalition or "axis powers") were the first countries to include human dignity in their constitutions following WWII (Shulztiner and Carmi, 2014). It was as if recent consequences resulting from dignity's absence were felt most acutely here and necessitated the realization and affirmation of this core moral premise.

Article 1 of the UDHR states that "All human beings are born free and equal in dignity and rights" (UN General Assembly, 1948). The weight of this statement is so profound and represents such a seismic shift in human consciousness according to Jonathan Mann (1998a) that, paradoxically, its importance is seldom appreciated. Dignity's position is similarly pivotal in numerous bioethical documents including the 2005 UNESCO Universal Declaration on Bioethics and Human Rights (UNESCO, 2005; Hofmann, 2020). Here, it is acknowledged by both advocates and critics alike as anchoring the principles of the document and providing its normative foundation (Have, 2006; Schuklenk, 2010). Irrespective of the validity of the document's criticisms, it is telling that the authors of an international, political, and bioethical instrument conceived dignity to be its guiding tenet.

Dignity has also been described as the "lifeblood" of the European Convention on Human Rights and invoked and appealed to repeatedly in legal rulings (Foster, 2011, 101). When its justifying position is carefully considered by those of the highest legal standing, it is simply acknowledged as Justice Munby did in the case R (Burke) versus General Medical Council, that "…the recognition and protection of human dignity is one of the core values—*in truth the core value*—of our society…" (2004, 21, emphasis mine). Dignity seems indispensable and insists on being acknowledged when it is being violated, as in atrocious cases of torture and rape (Bernstein, 2015). Explanations of the moral harm done in these cases are poorly satisfied with guiding moral theories utilizing elements such as autonomy, flourishing, achievement of pleasure, or absence of pain (Bernstein, 2015). Bernstein (2015) notes that a thicker, more complex conceptualization of human beings and their relation to others is required. To engage in discussion of atrocities with any depth of moral meaning requires a concept of sufficient gravity and breadth. This space is filled by the concept of dignity.

The persistence and ubiquitous nature of dignity do not prove dignity's normative position. The empirical does not dictate the normative. However, dignity's widespread presence and persistence communicate something about its position and nature. If dignity really encapsulates something foundational about what being human is and stands at the core of many moral decisions about human beings, then it comes as no surprise that dignity is found as central to a multitude of noteworthy ethical declarations and constitutions (Foster, 2011).

Indispensable nature

Difficult cases in bioethics present a further opportunity for dignity's foundational moral role to be made visible (Foster, 2011). Routine, "easy" bioethical issues can be dealt with by comfortably utilizing, for example, Beauchamp and Childress' four principles approach comprising autonomy, beneficence, non-maleficence, and justice (Foster, 2011). Difficult cases allow the essence of morality to be exposed by straining what is at the core of these moral decisions (Foster, 2011). When one listens to the moral grappling on issues of cloning, resource allocation, and the commodification of body parts, for example, there is often recourse to the foundational ethical conception of human dignity at the root of what matters (Harris, 1998; Foster, 2011). Here, a key understanding of what is valued in humans seems to insist on informing the ethical justification that guides our moral sensibilities. *Undermining the concept of dignity* is key to dismissing cloning objections, as Harris (1998) notes. The reason for agonizing over resource allocation is justified primarily by an acknowledgment of value belonging to a human being—something that grants a powerful sense of equality. Dawson et al. provide the "almost universally agreed upon…" justification for resource allocation as the notion "…that each person is due equal respect or is equally valuable" (2020, 751). Severing these qualities captured by dignity from other justifications such as resource maximization or redressing past injustices would result in moral reasoning that is simply insufficient or callous. Grappling with the essence of difficult moral cases allows the moral foundation of dignity to be displayed.

Let moral thinking be challenged by contemporary boundaries and it is dignity that is unearthed. The flippant use of body parts for example, elicits widespread moral revulsion. Foster's (2011) probing and horrifying suggestion as to the morality of a group of boys kicking a human head around a road appeals to the concept of human dignity to provide substance as to what makes this wrong. It can be argued that secondary principles such as autonomy, beneficence, justice, or non-maleficence, for example, are impotent to provide substantive normative rationales or do so in an unconvincing way.

The persistent and pervasive need for dignity is exposed when morality is pushed to its elemental form and indicates dignity's foundational nature. It is not an exotic flourish, but rather a recurring point on which a multitude of moral matters settle. Dignity fills a void—a foundational normative guiding position.

Dignity, with its ineradicable dimensions, is presented in this essay as the moral foundation and starting position when considering assisted dying. It is an alteration of dignity's form that *results* in the institution and acceptance of assisted dying in society. That is, when dignity's form is altered so that the long-held dominance of the basic dignity dimension over the subordinate personal dimension is reorganized, assisted dying results. Part III is a sociological description of how assisted dying together with forces that influence people are brought about as a result. In Part IV, I analyze the moral qualities of the altered form of dignity that gives rise to assisted dying.

The argument in Part II has asserted the categorical priority of dignity and substantiated this with a number of different reasons. Those who disagree may hasten to disregard what seems like a conditional argument. However, as an asserted a priori starting point, absolute proof of dignity's foundational position is not available. There must always be a starting position that is chosen and unguided by further criteria (MacIntyre, 2014). *Any other moral analysis of assisted dying must also have one*. The first step of theoretical formulation and understanding, the starting perspective infused with conviction, is sometimes one that may altogether escape notice (Wittgenstein, 1968). Hence, while I argue against assisted dying from a dignity perspective, I speak to those who are dismissive, too. Autonomy is similarly an overarching justification and starting point adopted by those condoning assisted dying (Batavia, 2000). For those supporting assisted dying, autonomy is often foundationally accepted as important and infallible. Hamalainen (2021) notes that when we approach moral issues, a lot may already have been decided. In Part IV, I argue that autonomy reflects a thin, attenuated expression of the dominant characteristic of the altered form of dignity—a dignity dominated by the personal dimension. Autonomy as a justification suffers from the same criticisms and failings as the altered form of dignity that gives rise to assisted dying. I find this altered form of dignity (and hence, autonomy) to be seriously deficient and ultimately self-defeating.

III. DIGNITY'S RESTRUCTURING AND ITS SOCIOLOGICAL REPERCUSSIONS

Individualism and the Restructuring of Dignity

The structure of dignity has been transformed. An alteration in the priority attributed to the dimensions within the concept has occurred. The dimensions cannot be co-dominant and direct morality without conflict (Pullman, 1996). Absolute inviolability of a human being cannot exist with absolute self-determination that demands otherwise. The unreserved preclusion of ending a life is incompatible with the unreserved choice to do so. Their nature, by definition, constrains the other. It is an alteration or metamorphosis of the concept of dignity that leads to the institution of assisted dying. With the metamorphosis, the dimension of the personal with its qualities of individualistic autonomy is given predominance over the dimension of basic dignity. The prioritized, weighted constraint that basic dignity has held both in the preceding centuries and within the understandings of Western society is negated. The *new dignity* with its reprioritized dimension now functions as the "moral" foundation and guide.

Individualism in Western societies seems responsible for the metamorphosis of dignity—the restructuring of the dignity concept. Modern individualism in the West has both grown and assumed dominance. Individualism as a guiding notion is the idea that the interests of the individual are or ought to be ethically paramount (Merriam Webster, 2023a). Individualism lays emphasis on the attributes, claims, and welfare of the singular—the individual within society (Celtel, 2004). Closely

associated with individualism is an intensification of the place and value of individualistic autonomy and the absolute priority given to self-determination. What is "right" simply becomes what is chosen.

The primacy of "me" together with the entitlements, normative standards, and goals this directs becomes the ordinary. This understanding constitutes our modern social imaginary—a way of seeing that enables one to make sense of the world and how society works (Taylor, 2004). It is not simply an idea we contemplate but rather a foundation *by which we think*; it is the way we imagine the societies we inhabit and sustain (Taylor, 2004). It is our background understanding that we come to share—our "collective consciousness"—that effects our moral orientation as a society (Coser, 1984, xvii).

A mistake may be to take this individualistic perspective for granted, to see this self-understanding as so natural that to see otherwise becomes impossible (Taylor, 2007). To live in a Western society as residents of this age is to be immersed in an individualistic way of understanding that is so all-pervasive that it becomes hidden from view.

The narrative of individualism in Western society elevates the individual to the central position from which individual freedom may be pursued. This comes with the increasing recognition of our rights to which we are "entitled" and which "liberates" us from the repressive and "irrational" moral ways of the past. This is the story and narrative that informs the social imagination in which we live (Smith, 2003). It communicates who we are and structures our reality—even defining what is sacred (Haidt, 2008).

The narrative of individualism's dominance has many likely contributions. John Stuart Mill's (1806–1873) harm principle is significant in this regard. It articulates that the *only* purpose for which power should be exercised over an individual is to prevent harm to others (Mill, 2017). While it is not Mill's complete position, it certainly is what has been extracted with far-reaching effects in advancing individualism (Jennings, 2009). Similarly, a legacy of understanding that has filtered down from Immanuel Kant (1724–1804) is a worth based on autonomy (rather than absolute intrinsic worth) interpreted as individualistic free choice and the respect of this quality (O'Neill, 2002; Byers, 2016; Saad, 2018). Rational autonomy was understood by Kant not as the capacity of unconditional free choice alone, but rather the capacity to self-legislate *according to and constrained by* the supreme moral law (Sensen, 2009). Dignity for Kant, *whether intended to or not*, is credited with providing a basis for human worth founded on rational autonomy and with this, an unconstrained understanding of this (Sensen, 2009). The erosion and decline of religious belief in the West would also appear to contribute to the accentuation of individualism (Taylor, 2007). Yet, individualism also appears to be a product of Christianity—the dominant form of religion in the West (Buss, 2000; Taylor, 2007). With an intense focus on Jesus Christ, a heightened consciousness regarding the uniqueness and personality of the individual was created (Taylor, 2004). Either way, religion's effect on the ascendency of individualism seems significant (Buss, 2000; Taylor, 2007). Finally, it has been suggested that the development of a pluralistic society itself accentuates individualism (Weston, 2014; Lukes, 2014; Durkheim, 2014). The necessary common shared appreciation of the sacred is maintained by prioritizing the individual (Weston, 2014). The need for social cohesion remains, and the consensus belief that maintains this is "…reduced to the cult of the individual," as Émile Durkheim (1858–1917) described (2014, 317).

The rise and dominance of individualism in the West may be derived from these and other sources—the *exact* influence and contribution are ultimately irrelevant for the purposes of this dissertation. There is no single history accounting for individualism's ascendency. What is important is the fact of individualism's dominance in the West, which has allowed for a restructuring of the concept of dignity. For, until recent history, the dimensions of dignity have been understood as giving precedence to basic dignity (Pullman, 1996, 2010).

Evidence for individualism's pervasive and dominant presence may be found in historical and contemporary literature and academic fields, while personal experience testifies to its reality. Durkheim's (2014) landmark book—*The Division of Labour in Society*—is a carefully justified explanation of the rise and dominance of the individual and their autonomy which is elevated to a sacrosanct position (Coser, 1984). Similarly, Taylor in his monumental work *A Secular Age*, recognizes processes within the history of Western civilization which contribute "…towards that primacy of the individual which defines our culture" (2007, 94). These are but two examples of works that portray the rise and reality of individualism.

In the academic field of bioethics, Fox, Swazey, and Watkins in their book *Observing Bioethics* note the "…primacy that the field has accorded to a conception of individualism and individual rights that

emphasizes the values of individual autonomy and self-determination…" (2008, 153). Those familiar with the field will be aware of the common theme of the dominance of individualistic autonomy within it (Gillon, 2003; Dawson, 2010; Saad, 2018). Relational autonomy as advanced by feminists in many respects emerges and even *defines itself in distinction to this* (Mackenzie and Stoljar, 2000). Relational autonomy, *in contrast* to individualistic autonomy, sees the person as socially embedded and interdependent and challenges the individualist framework (Mackenzie and Stoljar, 2000).

Finally, the dominance of an individualistic outlook may be simply appreciated by the reflective individual living in Western society today. I appeal to the reader's experience. From a hyper-individualistic culture that frames life goals and decisions as solely individual in nature to an emphasis on and pervasive demand for choice and rights, a "me-culture" is easily recognized. From proffered ways to assure your "best life now," a narrative is told and reiterated with the singular "I" and "me" as the consistent and central theme. This individualist conception saturates Western society.

Dignity's very essence has changed as a result of individualism's influence. Just as in biology, the tertiary structure of a protein comprising the shape and orientation of its parts is essential to what a protein is and does, so the reorientation of the parts of dignity reorders everything. This may go quietly unnoticed. The possessors of dignity—people—are understood differently. People are reimagined. With the reorientation of dignity, the bearers of this core value undergo a remarkable change. Humanity sheds the category of sacred. People as a category are no longer primarily considered "sacred" per se—no longer regarded as too valuable to be interfered with for what they are (Oxford Dictionary and Thesaurus, 2007c, 911). The sacred no longer primarily resides in its bearer, but rather in the respect and exercise of the bearer's choice.

Elevated and reinforced, individualism reshapes the moral concept of human dignity. Dignity as a concept is restructured. The priority of the personal dimension becomes of paramount importance within the concept. The concept is reweighted. The supremacy of the individual, personal dimension alters the structure of dignity and moral decisions that rest on this understanding. Dignity as a restructured concept is depicted graphically in Figure 2.

Assisted Dying: Conception to Birth

The power that brings assisted dying to life lies in the refashioned dignity concept. Before deciding about moral problems, there is a conception of what is morally important and what is taken for granted (Ives and Dunn, 2010). Once the metamorphosis of dignity is complete, the birth of assisted dying legislation is its practical outcome. With a reorganization of the preeminent dimension of dignity to that of the personal, reception of one of its children is only natural. The new understanding of dignity captures the priority of noninterference. This is the highest value, the core of the reorientated dignity. Dignity succumbs to the individual dimension and simply the pursuit of "…own good in our own way, so long as we do not attempt to deprive others of theirs or impede their efforts to obtain it" (Mill, 2017, 24). When this understanding is embraced by a sufficient societal majority, the time for the outworking of the idea has come. As the value central to those who support assisted dying is individualistic autonomy (Batavia, 2000), assisted dying emerges as its legitimate child.

Within a few short decades, assisted dying thus changes within society from something generally not able to be contemplated to something that is intelligible. It becomes sufficiently normalized to be birthed. The new dignity not only accommodates assisted dying but rather commends its existence. Abstract changes in moral concepts, as MacIntyre (2014) so astutely recognizes, are always embodied and become manifest in real, particular events. Assisted dying is no exception.

The relation between law and morals is brought into focus here, for it is legalization that formally delivers assisted dying. Law is necessarily coupled with morality. This contrasts with the legal positivist who accepts that morals and law may coincide, yet who denies a *necessary* connection (Coleman

a. Personal dignity dimension
Basic dignity dimension

b. Basic dignity dimension
Personal dignity dimension

Fig. 2. Dignity as a concept is depicted with its necessary dimensions in both a and b. The dominant dimension of the dignity concept at any one time occupies the foundational position below the line (in bold). The long-held concept of dignity is shown in "a." The new, altered concept of dignity is shown in "b."

and Leiter, 2010). Law cannot stay removed from moral foundations. Law *reflects* the morality of a society; moral commitments are *concealed* in law. When the concept of dignity changes, its moral orientation changes and this is reflected in law. Ronald Dworkin (1986) describes how challenging cases in law demonstrate an alignment with accepted morality—emphasizing the existence of a necessary coupling. Legal rulings are more than the application of rules; they are a moral interpretation that draw on principles that rely on a "moral vision" (Bix, 2010, 223). The legal rulings are constructed in that they are built on principles that, in turn, rest on moral foundations that inform them (Bix, 2010; Beever, 2021). The judgments in law are constructed by humans who have developed, exist, and function in society as moral and mutable beings.

Assisted dying is introduced into law because societal morals dictate its acceptance. In Western Australia, the parliament appointed a committee to inquire into the need for laws regarding end-of-life matters (Western Australia Parliament Legislative Assembly Joint Select Committee on End of Life Choices, 2018). The resulting report that would be used to direct strategy (aptly named "My Life, My Choice") encapsulated recommendations consistent with its title (Western Australia Parliament Legislative Assembly Joint Select Committee on End of Life Choices, 2018). It was "…broad community agreement regarding individual autonomy…" that formed the basis for the committees' recommendations to introduce voluntary assisted dying (Western Australia Parliament Legislative Assembly Joint Select Committee on End of Life Choices, 2018). It is clear that a conceptual reorientation had occurred. An altered concept of dignity—one that prioritized the individual, personal dimension—fostered an individualistic autonomy that had the concern of choice as its central and dominant concern. Those legislators as members of society were likely both holders of this sentiment and facilitators of the collective sentiment. Those in positions of government are authorized interpreters of the collective sentiment in a democratic society (Durkheim, 2014). The idea of assisted dying and its subsequent birth results from a conceptual reorientation within society completed by the introduction of legislation. The enactment completes the idea, reinforcing and legitimizing this in law and in the societal mind.

There is a corollary to the acceptance of the assisted dying institution with its foundation. This is the ineradicable accompaniment of the understanding and affirmation in society that some people have lives that are no longer worth living and would be better off dead (Keown, 2012). The person who chooses assisted dying and those condoning it necessarily affirm this. The person asserts that they will be better off dead; those condoning their choice accept it. A justificatory premise of sufficient satisfaction and sway *must* be present to permit this. Without this, the taking of human life would simply be impossible to accept. Consciously or subconsciously, a justificatory premise must inform the "morality" of the institution of assisted dying. The new dignity performs this function.

An alteration in prohibited social practices—of which assisted dying is a representative—may be considered in a number of stages. The first stage is one in which the context provides intelligibility (MacIntyre, 2014). The reason for the prohibition is consistently aligned with an underlying moral premise. The second stage is one in which the societal practice is decontextualized. The premise, which forms the basis for the societal practice and gives it meaning, is altered. Prohibitions remain, yet are detached from and remain inconsistent with their guiding moral foundation (MacIntyre, 2014). A third stage occurs when the moral prohibition is abolished as a result of *realignment with an altered premise*. Here, assisted dying regains consistency with its informing moral premise—being aligned with the restructured concept of dignity.

Normalization

While the institution of assisted dying is passed into law in a variety of countries and states, this occurrence is never without some initial opposition and contention. It tells of the dynamic of assisted dying *normalization*. Normalization may be understood in two ways (MacKellar, 2017, 219). "Normal" in the first sense means what is regular, usual, typical, or conventional (Merriam-Webster, 2023b). It is what is accepted practice (Snijdewind et al., 2018, 295). In the second sense, "normal" also means conforming to a type of standard (Merriam-Webster, 2023b). "Normal" here is really an issue of calibration.

In the first understanding of becoming more ordinary in society, assisted dying normalization occurs on a progressive continuum, commencing prior to and contributing to its legalization. After

legalization, assisted dying continues to become increasingly typical or conventional, although now it is the concept of assisted dying together with its practical outworking that is accepted as increasingly usual. Several factors may contribute to this form of normalization. Positive factors act to encourage the normalization process and may include time, geographic location, and increasing acceptance (MacKellar, 2017). The established nature of assisted dying over a prolonged period of time contributes to its perception of becoming typical or usual (MacKellar, 2017). Similarly, the geographic proximity of one jurisdiction to another where normality has been established is also likely to play a positive role (MacKellar, 2017). This is suggested to have occurred in Belgium, Netherlands, and Luxembourg—the closely knit "Benelux" countries where assisted dying legislation in one country influenced that of the others (MacKellar, 2017, 221). Similar patterns may be seen between states in Australia where the sequential challenge and subsequent adoption of assisted dying law occurred in a number of states in a relatively short space of time (Okninski, 2023). An additional positive factor contributing to the normalization process is acquaintance and familiarity with people undertaking assisted dying (Snijdewind et al., 2018). Such open communication within members of society promotes familiarity and contributes to what is viewed as regular or typical (MacKellar, 2017).

Negative factors, in contrast, may inhibit the normalization process and prevent assisted dying from being understood as typical or usual. A community holding strong religious traditions may provide an example of a negative factor. Through religious convention or reasoning, assisted dying is not accepted as typical or normal.

Apart from imparting raw data, media communication is a means of forming and sharing understandings—a way of altering and maintaining cultural beliefs and acceptability over time (Simonson, 2002). It has been noted in Canada, for example, how media representation was weighted toward positively portraying the impact of assisted dying introduction in the health system (Herx, Cottle, and Scott, 2020). Incorporation of personal stories and portrayal of a "victim" status in need of rescue from a deteriorating body act as positive and emotive communication tools (Seale, 2010, 862).

Normalization in the second sense occurs when assisted dying demonstrates an increasing conformity to the reconfigured dignity which serves as the standard (MacKellar, 2017). Introduction of assisted dying law aligns practice with its conceptual standard and so normalizes it. A society *not* adopting an assisted dying law, yet holding a conception of dignity with a primacy of the personal dimension, exists in a state of tension due to inconsistency.

The degree of normalization of assisted dying in both these understandings—that of becoming conventional and as relating to a standard—are by their nature underdeveloped yet sufficient at the time assisted dying is affirmed within law. Opposition and contestation of assisted dying laws are an inevitable result of its state of maturation as a concept normalizes within a society. This tension will lessen with normalization which removes the source of such discord. Normalization of assisted dying is really an anticipated process as a society experiences the practical outworking (assisted dying) of a more discreet and more fundamental cause.

Assisted Dying and Accompanying Forces of Influence

When people are understood as morally significant according to a reprioritized dignity, an environment not previously encountered is brought into effect. It is an environment with forces that did not previously exist. The reimagined person—conceived according to the new dignity—is made vulnerable. The new dignity marks the loss of the constraining effect and protection of the inviolable worth that the priority of basic dignity affords. With assisted dying laws, a new environment is created. It is an environment that contains both a parochial understanding of harm and an inflated confidence in the wisdom of unbridled autonomy. Any protection from these forces now needs to be cobbled together, formulated in law, shaped according to the needs of the time. Three forces of influence that accompany the introduction of assisted dying include: (1) the force of legitimization; (2) forces of duty; and (3) economic forces. These are described below.

The force of legitimization

The force of legitimization exists as a pressure of influence resulting from *possibility*. It affirms, condones, and redefines what is accepted. The interference to one's plan is removed; persons are no

longer deprived of their way (Berlin, 2002). It is a freedom resulting from the removal of obstacles. In this sense, it is a force resulting from what Isaiah Berlin describes as a "negative freedom" (2002, 169–177). Societal legitimization justifies the possibility of assisted dying and supports it as permissible. It exists as a force by acting on a predisposed person, moving their intention towards the resolution of assisted dying; it is synergistic with a person's resolve.

Communication is central to establishing this force. Communication, as Simonson (2002) explains, is both *transmission* and *ritual*. As transmission, knowledge in the form of data about the availability, criteria, and processes of assisted dying is specified (Simonson, 2002). Communication as a ritual reiterates the values and a particular narrative frame or common understanding (Simonson, 2002). Communication "…does more than neutrally report on the world; it also recreates, reenacts, and sometimes changes the world" (Simonson, 2002, 35).

From the ritual perspective, the communication of assisted dying is a reiteration of its acceptability within a culture, reassuring its permissibility while removing any stigma that may remain. When communicating assisted dying as a ritual, acceptability is presented and cultural norms are affirmed (MacKellar, 2017). Ritual communication even exists in the language used (Parkinson et al., 2005; Kirchhoffer and Lui, 2021). Language such as "assisted suicide" and "euthanasia" is avoided because it carries negative connotations (Kirchhoffer and Lui, 2021, 101). Communication as ritual speaks to a society about assisted dying—legitimating it by celebrating it as a means of hope and removing any negative connotations associated.

Furthermore, assisted dying is affirmed as an extension of individual control and something to be celebrated (Mann, 1998b; Cunningham, 2019; Perpitch, 2019). This satisfies popular Western culture that is now comfortable with the dominance of the personal, individualistic domain. The language of compassion is communicated, cloaking the core issue, which is ultimately a supremacy of choice. For the response to suffering has not been lacking up until the moment of assisted dying introduction. If this were at the heart of assisted dying, there would be no qualms to its introduction for all forms of suffering at all ages.

Thus, the introduction of assisted dying brings with it a new environment with a force of legitimization. It is a force that exerts influence on decisions by way of reaffirming acceptability and modifying the cultural consciousness from which the person interacts and uses as a substrate for decision-making. It is a force that acts on exposed persons, making them more likely to utilize assisted dying because of its perceived permissibility.

Forces of duty

In a poignant letter contained on her webpage, Gillian Bennett (2014) provided her rationale for "committing suicide." "…[I]t matters to me not to become a burden to them, or to Canada" wrote Bennett (Bennett, 2014). She details the progressive symptoms of her dementia yet her current capacity to assess her situation. Her letter contains justifications regarding her decision. Interestingly, these both relate to her immediate family while also showing repeated concern about the cost to her country and even the world. "The world strains under the weight of an aging population" (Bennett, 2014). According to Bennett (2014), "…by 2045, the ratio of working-age citizens to their elderly dependents will become increasingly burdensome…." Burdens resulting from the elderly also squeeze dry "…grandchildren's schooling, their educational, athletic and cultural opportunities" (Bennett, 2014). With these reasons, Gillian Bennett completed these words before acting in accordance with the force of duty seemingly expressed in them.

The force of duty is relational in origin. While duty is made possible by the legalization of assisted dying, this particular pressure is born and varies largely according to the existence, strength, and nature of interpersonal relationships. It originates from a sense of responsibility, obligation, and attachment to others and acts as a complex motivation for an individual's decisions and their out-workings (Mann, 1998b). Assisted dying brings with it pressures that act on people causing them to "choose" to die (Mann, 1998b, 25). Duty moves a person to do something because it is perceived to be right (Oxford Dictionary and Thesaurus, 2007a). This force also appears prominently among people who are disabled and grappling with assisted dying law. Tonti-Filippini, a bioethicist who lived with a disability, expressed that if euthanasia were lawful, there would be moral pressure to make an end of a burdensome life (Jones, 2017).

We live in a dynamic social fabric comprised of a multitude of relationships and these act as substrate for the often poorly appreciated complex social forces of duty. We may be parents to deeply loved adult children who themselves are caught in the responsibilities of nurturing a growing family with divided time and competing interests. Some comprise members of an extended family balancing work and career development while accommodating to life's financial pressures. We may be a spouse stretched with the demands of care for another while also accommodating a new diagnosis amidst the relentless strain of chronic disease. Others are proud grandparents to curious and eager grandchildren who rightly yet obliviously and exhaustingly explore their new world, engaged in the "…splendid miracle of being young" (Hugo, 2016, 1233). A myriad of relational webs with nuanced characteristics exists. For many, it is care, concern, and love that form their common bonds. The notion of autonomous decision-making, immune from nudges and pulls from a variety of social pressures, is misleading, simplistic, and blind (Mann, 1998b).

It is here, amidst the busyness and messiness of life, that the force of duty is born. The force of duty understands sacrifice, the inconveniences of a dying process, the burdens that are necessarily placed on others, and the reasonable desire to allow others to move unhindered in their lives (Mann, 1998b; Gastmans, 2017). We feel and are moved by pressures that are born from the stuff of life. Gillian Bennett (2014) wrote of these in her decision framework. Choice is not carried out in a vacuum (Jones, 2017). While the most exceptional and noble among us may be driven by the more distant relationships to a country or the world, the ordinary may arguably feel this force most acutely as a result of those people closest to us.

Assisted dying, fueled by the force of duty, may not only be acceptable but even preferable and praiseworthy. Driven by duty, assisted dying makes sense as a means to achieve a good end while limiting inconveniences and costs to important others (Mann, 1998b). Moved by the force of duty, assisted dying becomes rationalized. In short, the force of duty has its reasons, and assisted dying, once rationalized, presents a rational solution.

Economic forces

Economic forces—pressures created by economic considerations and consequences—are a third group of forces brought into existence. An environment with market pressures that were previously prohibited is opened. The person is moved to decide in a particular direction because of economic concerns. The immunity conveyed by the priority of basic intrinsic worth is negated. In comparison to forces of duty, economic forces are not dependent for their existence on relationships and the social ties that undergird these. Of course, economic forces may co-exist with forces of duty such as when pressures of responsibility to family members are intertwined with cost implications. In their pure form, the ties that bind the person to economic pressures exist because of economic consequences alone.

A former health minister in the United Kingdom commented that "…it is an inescapable fact that ultimately policy is driven by money…the stark fact is that palliative care is expensive and the lethal pill is cheap" (Keown, 2012, 103). Fuelled by economic prospects and conveniences, pressures may be exerted in a variety of manners by organizations or governments, and these provide examples of economic forces operating at a macro level. Jones (2017) notes how Tonti-Filippini—as someone living with a debilitating chronic illness—experienced the force of this economic pressure when receiving an annual letter from his health insurer outlining his total cost to the fund. Humphry, as an advocate of assisted dying, identifies the power and priority of economic pressures when he predicts that "…in the final analysis, economics…will drive assisted suicide to the plateau of acceptable practice" (Keown, 2012, 138).

At the micro level of the family and community, economic forces may also come into play. In an environment where assisted dying is acceptable, the calculus of cost on families and the "cost-effective solution" do not go unrecognized (Mann, 1998b; Gastmans, 2017). While calling attention to economic forces may seem crass and taboo, it is worthwhile remembering that this is largely because of perceiving others according to a form of dignity prior to its reordering and legitimization. With normalization, economic forces at play simply become realities, and their open recognition merely represents a cost-effective, logical, and even considerate response.

Shaw and Morton, in *Counting the Cost of Denying Assisted Dying* (2020), provide a number of candid economic arguments for permitting assisted dying. These include greater economic benefits to society

from reduced resource use and from potential organ donation. Shaw and Morton note that these are additional arguments and not primary rationales for assisted dying, yet are anxious to stress that they "… should not be neglected when considering the ethics of assisted dying" (2020, 65). Inextricably intertwined with the legalization of assisted dying is the reality of economic concerns and the forces derived from them. When assisted dying is introduced, an environment ignorant of price—a "kingdom of ends" (Kant, 2019, 4:434)—is destroyed. Economic forces are *unavoidably* brought into being with assisted dying.

IV. THE MORALITY OF ASSISTED DYING

Assisted dying has been appraised in sufficient depth and justified as unfolding from a particular interpretation of the concept of dignity. With this perspective, the morality of assisted dying may be analyzed. This is *not* to assess assisted dying's morality merely according to societal rules and expectations embodied in societal norms or *mores* (Blackford, 2016). Norms establish the rules and justify the moral system at a point in time. They are the prevailing "morality" and will always appear ultimately "true" to those who have been socialized into it (Blackford, 2016, 10–23). Assisted dying follows as "justified" and "moral" in a society having an entrenched conception of dignity with a form favoring the personal over the basic intrinsic dimension. The opposite will be true of a society grounded on the alternate form of dignity. However, there is available reasoning that may infer the morality of one form of dignity over the other. First, the form of dignity that serves as a justification for assisted dying is deficient. Second, assisted dying rests on a moral justification that is self-defeating.

Deficiency of a Justification

Assisted dying originates from a guiding form of dignity that prioritizes the personal dimension. The contemporary assisted dying debate whittles down this justification to that of "autonomy," where autonomy is an individualistically understood one (Batavia, 2000, 550). This is really a limited view of dignity that arises from the personal dimension of dignity alone and that fails to even recognize the comprehensive concept from which it is drawn. The new, altered form of dignity that prioritizes the individual is, of course, not autonomy. Rather, this form of dignity intensely stimulates and gives expression to autonomy with the same hyper-individualistic characteristics. The dominance of a personal dignity dimension—focused and upholding all things of the individual as of prime importance—gives rise to an autonomy that is similarly motivated. Those dismissing the framework of dignity as the moral foundation are really just using a reduced nomenclature and understanding—an *impoverished* form of a restructured dignity. The altered power of dignity with its governing personal dimension powers the usual assisted dying justification of autonomy. The characteristics of the dominant personal dignity dimension are all that is seen, and it is simply referred to and understood as "autonomy."

The problem with assisted dying is that its moral justification—a reorientated dignity concept or simply its approximated derivative (autonomy)—is seriously deficient. While it values the individual's choice per se as unconditionally moral, it needs to ensure that it results in a choice that is right and so validates itself as moral. It fails to do this. Assisted dying in contemporary society is based on an autonomy premise, and there are good reasons to notice that this is a myth. The autonomy justification does not actually meet its own criteria. This needs to be taken seriously. Autonomy is upheld as a sufficient justification of assisted dying by its proponents by: (1) ignoring the inadequacy of voluntariness when faced with the choice of assisted dying; (2) placing undue faith in safeguards; and (3) dismissing forces that may influence people.

Voluntariness

Voluntariness enables meaningful choice when undertaking life-authoring decisions and is a necessary component of autonomy (Colburn, 2020). Voluntariness implicitly consists of reasoning that is adequately untainted by influence. Yet, voluntariness also has a dubious quality (Kamisar, 1958). The *failings of voluntariness* are correctly perceived by situating the notion of a socially stripped, isolated person back into their unavoidably relational environment while honestly acknowledging cognitive failings. It is to understand people as human beings again.

The illusion of sufficient voluntariness for the decision of assisted dying is achieved by conceiving of a person according to a notion of unmarred individualism. The individual is understood as removed from the sphere of influence in which they necessarily exist. In reality, we are deeply and

unavoidably influenced by others, even if we neglect to recognize it. Our reasoning is inextricably socially bound. No person makes decisions as the solitary, detached person within a void. The illusion of sufficient voluntariness disregards our unavoidably intertwined natures, our social connections, and their effects. Even the recluse communicates powerfully with others.

Any conception of autonomy that fails to incorporate or appreciate the true extent of interpersonal and social influence is simply, as Donchin (2000) notes, an illusion. An individualist-prioritized framework is overly narrow and fails to recognize the powerful forms of influence that social structures and institutions may impart and how these may shape people's decision-making process (Ho, 2008). We exist as the persons we are because of our connections and obligations to others. We are socially embedded and irreducibly relational—our motivations and reasons are both influenced and altered by our social context, including the people with whom we interact and the society in which we live (Christman, 2014). Not only are we susceptible to influence because of our social natures, but also because of our cognitive frailty. A broad range of literature from the behavioral and social sciences plainly tells us this (Kahneman, 2011; Conly, 2012; Kahneman, Sibony, and Sunstein, 2021; Thaler and Sunstein, 2021). We succumb to a myriad of cognitive deficits and biases which are not only pervasive but also affect the best of us (Conly, 2012). We miscalculate and erratically undervalue or overvalue information while simply being careless and markedly prone to assumptions (Conly, 2012). Alarmingly, we fare no better in the face of life-critical decisions (Conly, 2012).When assisted dying is allowed in a society, it is done so by brushing aside our innate cognitive frailty and our vulnerability that unavoidably exists; it chooses to disregard all this for a fictitious view of people and how they make decisions.

An argument for assisted dying emerges when concerns about voluntariness are suppressed. *It may then be objected that death is not always a harm but rather suitable in the face of sufficient suffering, and assisted dying is therefore an appropriate response* (Wiebe and Mullin, 2023). Apart from this reasoning entailing self-fulfilling inclusivity, what makes it wrong may altogether escape Western moral discretion that is so often calibrated according to utility. The "solution" of death *in the form of assisted dying* should not be an available response. It is not that death is *always* a harm. It is that assisted dying approaches a person with the wrong posture and "remedy." A suitably *human* response requires even more extensive engagement with suffering—established on a care, affirmation, solidarity, and commitment to journey together rather than on negation, annihilation, and killing. Suffering requires a response that challenges our values and approach that is dominated by functionality and efficient control (Bishop, 2011).

This aside, the objection remains tethered within a system of thought that is unrealistic. Its reasoning *demonstrates that it believes* in the isolated individual and is insensitive to the import of social connection, deficiencies of voluntariness, and the influence on others. As an objection, it is a thought experiment that exists only as fiction, because it does not transfer to reality. Even if death were a lesser harm to a particular individual and assisted dying an appropriate solution to their suffering, its existence generates an environment of harm for others—something that such a system, *by its very nature*, dismisses.

Disregarding autonomy's authenticity because of flawed voluntariness may seem excessively stringent and unrealistic. The traditional liberal may support the sufficiency of voluntariness for the majority of autonomous decisions in life, even while conceding that most choices fall short of the ideal (Feinberg, 1986). This seems a reasonable position. However, there is a limit to this and this is set by what voluntariness is used for. According to the traditional liberals' suggested standard, *the greater and more irrevocable the harm risked, the greater the degree of voluntariness required for it to be permitted* (Feinberg, 1986). If this is the case, *there is no situation in which greater voluntariness is required* than in assisted dying. Voluntariness and hence autonomy is not up to its alleged ability in this situation; it is deficient. The trouble with assisted dying is that it sets back a person's interests in the most definitive of senses—it destroys their future possibility and the ability of revision. The irrevocable nature of assisted dying means the voluntariness of the "autonomous choice" is never verifiable. The significant nature and finality of harm precipitated would comparatively require an *unassailable voluntariness*. Those advocating for assisted dying, then, are inconsistent by erroneously attributing sufficient voluntariness, even according to their own standard. Possessing sufficient voluntariness and hence autonomy for an assisted dying choice *does not exist for anyone*. These problems with voluntariness and autonomy ground a normative perspective. Since autonomy has serious deficiencies and the

impossibility of attaining an adequate standard, it is wrong for the advocate of assisted dying to rely on autonomy to do moral work. Autonomy is impaired and is insufficient to justify assisted dying as a right.

Safeguards

Proponents of assisted dying hold fast to the sufficiency of voluntariness—even in the circumstance when it is to be doubted (when irrevocable harm is risked). Insufficient voluntariness for an assisted dying choice is not acknowledged—at least not formally. However, the problem with voluntariness that ineradicably remains is "dealt" with by instituting safeguards. Safeguards are also termed *procedural guarantees* (Raus, Vanderhaegen, and Sterckx, 2021, 82), *prevention measures* (Pereira, 2011, e38), or *filters* (Jackson, 2012, 50). The purported function of safeguards is to ensure that requests for assisted dying are voluntary and well considered by the patient (Emanuel et al., 1998). Their presence acknowledges a precursory environment of potential harm and the forces of legitimacy, duty, and economics that are born with assisted dying. They guard the safety and security of vulnerable persons and so seek to prevent abuse and limit risk (Schuklenk et al., 2011). Safeguards are said necessary to "…avoid 'false positives' to as great a degree as possible" (Schuklenk et al., 2011, 49–50).

Two broad groups of safeguard strategies exist supposedly to limit the risk that assisted dying law poses to society. The first restricts and clearly *demarcates* the population of persons exposed to coercive forces. Eligibility criteria constitute this group (e.g., greater than 18 years; diagnoses that cause death within a certain timeframe and the character of suffering—that which is intolerable to the person). The second group includes strategies to limit "inappropriate" or coerced reasoning of the demarcated population. Examples of this include time-period requirements between first and final requests, the involvement of multiple and independent medical practitioners, psychiatric assessments, and reporting/reviewing procedures, to identify but a few (Jackson, 2012; Onwuteaka-Philipsen, Willmott, and White, 2019; Raus, Vanderhaegen, and Sterckx, 2021). All safeguards in effect operate through these management strategies. Safeguards are intended to be strategies acting to manage the environment (with its coercive forces) introduced by assisted dying and implemented to mitigate risk of abuse.

The boundary of the demarcated population is not, however, dependent on safeguards, as may superficially appear to be the case, but is, rather, reliant on the more powerful and foundational moral premise of dignity. At any one time, a line in the sand is formed when an equilibrium is reached. *The equilibrium is dependent on the outworking of the altered concept of dignity limited by the rate of normalization within a society.* The population-determining position of this line *does* consist of safeguards; however, their function is a descriptive and reassuring one. The "eligible population" is the group of people who, according to the current societal law, qualify to choose to assisted dying. Safeguards *describe* the momentary border of the eligible population. They reassure by presenting a *tightly regulated* and *protected system and so provide* the "correct" cultural script for acceptance at that point in time (Karsoho et al., 2017). The impermanent, malleable nature of safeguards in a society over time and their inconstant stringency bears witness to their true function. Historic examples are seen in Belgium and the Netherlands, while Canada more recently displayed this dynamic as to how safeguards change (Van Gool and De Lepeleire, 2017; Health Canada, 2022). They do not simply and randomly change; the change is in response to an underlying impetus.

The altered conception of dignity is the impetus that leads to the expanding eligible assisted dying population. Society acts consistently and in accordance with its moral foundation. It is only the gradual nature of societal acceptance that delays the extent of its application. *Expand* means to become larger or more extensive, to increase in size/scope (Oxford Dictionary and Thesaurus, 2007b). Initially confined to adult terminally ill patients, for example, the population expands to include those with chronic but non-terminal illness, children, those with mental or existential suffering, and those suffering without the capacity to choose (Herx, Cottle, and Scott, 2020). Proponents and providers of assisted dying may dismiss or reject this expansion by claiming that what is happening is rather "…a maturing of the understanding…" of what is being done (CBC Radio, 2019). They are incorrect in the technical sense of "expansion," for this extension is borne out in the facts. The existence of an expanding class of persons eligible for assisted dying bears expansion out as an empirical fact (Koch, 2021). Before a particular date, assisted dying did not

apply to population X; after such a date, it did. They are, however, right in a theoretical sense. Once the legalization of assisted dying is permitted, simply on the basis of consistency it will re-configure on the basis of its foundational moral premise. Its permissibility to a whole range of persons within a population is *correct* as a theoretical concept from the moment of its legalization. It only needs progressive realization. This progression occurs at a rate that normalization within a society allows. "Maturing" perhaps inadvertently captures this process quite accurately, for assisted dying is present in a conceptually whole form from the time of its introduction in society. It awaits only to attain its developed state. Safeguards as dependent variables will configure and shift accordingly. This is evidenced in countries instituting assisted dying and continues to do so (Van Gool and De Lepeleire, 2017; Health Canada, 2022).

Dismissed forces of influence

A third manner in which proponents uphold autonomy as sufficient for an assisted dying choice is by dismissing the existence of forces that may influence people. Conceptually, forces or pressures may be thought of as existing on a spectrum (Feinberg, 1986). Compulsion lies at one extreme with coercion, manipulation, persuasion, enticement, and simple requests on the other (Feinberg, 1986). Compulsive pressures describe a person being compelled to act because the alternatives are difficult in the extreme or even near impossible (Feinberg, 1986). Driving into a group of people rather than over a cliff because of fear of death when a car's brakes suddenly fail provides an example of compulsion (Feinberg, 1986). Coercive pressures differ in that they do not compel but rather destroy the appeal of the alternative (Feinberg, 1986). The pressure on landowners to accept inadequate relocation reimbursement from a chemical company, which has produced nearby harmful waste is an example of coercive pressure that detracts from voluntariness (Feinberg, 1986). The forces continue on this presumed spectrum from greater to lesser influence respectively.

Proponents of assisted dying, with rather concrete thought and imagination, set a high threshold for the significance of forces and their influence on voluntariness (Feinberg, 1986). Our voluntariness, however, is not only influenced by compulsive and coercive forces in the extreme. The presence and influence of the forces of legitimacy, duty, and economics have been outlined. We should not be so naïve as to dismiss the profound effect that a range of forces may have on the outworking of our choices. John Stuart Mill was rightly impressed by subtle but even more powerfully influential forces than those on the extreme of the scale. Mill noted that "…society can and does execute its own mandates…it practices a tyranny more formidable than many kinds of political oppression…it leaves fewer means of escape, penetrating much more deeply into life…"(2017, 10).

Assisted dying relies on autonomy as its core normative foundation to justify it as morally correct. Rather than being a secure and robust justification, it is deficient. Its voluntariness is a myth and a range of coercive forces unavoidably exist, while safeguards are impotent to protect against any deficiencies in autonomy that may be acknowledged. Safeguards are formulated and reformulated—shifting according to an underlying priority. When a society disallows assisted dying law, it is an act of paternalism. Paternalism interferes with the autonomy of a person, without their consent and for their good (Dworkin, 2020). *Hard paternalism* interferes with choices that are fully voluntary, while *soft paternalism* interferes with choices that are non-voluntary (Dworkin, 2020). Mill's (2017) example of a man who wants to cross a bridge, but who is unaware of an inherent danger and is thus stopped against his will from crossing is an example of soft paternalism. Disallowing assisted dying is an instance of soft paternalism because a deficient autonomy is never adequate for a death choice. "Voluntariness" within a milieu of influence and in the face of irrevocable harm is wholly insufficient for a decision about death.

If a legalized institution were to cause harm of the most severe kind to a number of members of a society—say three or even one out of every 100 persons—it would be considered as a moral wrong. These statistical musings may be a gross underestimate. Occasionally, the legalization of assisted dying law is retracted because of realized instances of impaired voluntariness. The Northern Territory in Australia provides one such example (Kissane, Street, and Nitschke, 1998). However, this realization, admission, and retraction are rare.

Perceiving people's vulnerability and the inadequacy of autonomy is itself dependent on a precondition. The insufficiency of the justification of autonomy in a society may simply be dismissed

as if non-existent. A society enamored by individualism will uphold an individualistic autonomy as obvious and will be blind to even seeing its intrinsic inadequacies. Kamisar, in the landmark assisted dying debate in 1958 with William Glanville, saw that there was ample reasoning for the incapability of an assisted dying choice (Kamisar, 1958). It was, rather, some other condition that acts as the background and is responsible for driving assisted dying into effect. The real background condition is dignity itself. It is the form of dignity with its prioritization of the individual dimension *that disallows the significance of moral harm to be perceived at all*. A dignity weighted by the absolute dominance of the personal elevates the voluntariness of autonomy to that of the infallible. Social connectedness and influence are dismissed. The bar on the spectrum of forces that sets the mark for what counts as a significantly coercive force is similarly set very high. Very few influences apart from compulsion and perhaps some coercive forces are *seen* to matter. The absolute deficiency of assisted dying's justification is ignored.

Self-Defeating Nature

Apart from assisted dying resting on a deficient justification, there is a further feature of the dignity from which it is derived that marks it as immoral. Assisted dying originates from a form of dignity that is self-defeating. Without the primacy of basic dignity, any significance and consideration the individual may have becomes illusory and will fade, *having no foundation*. Without the primacy of basic dignity, the respect owed to the individual, personal dimension is lost. We need a conception of human beings that has the belief in and categorical priority of being human per se. A dignity dominated by the individual dimension is a form that is paradoxical: while initially giving priority and eminence to the personal, it lacks any good reason for this and over time negates the person. This form of dignity self-destructs. Morality originating from dignity that neglects the primacy of the basic, intrinsic worth dimension is *self-defeating*. Without assigning priority to basic dignity, the value of a person may or may not be relevant and may or may not be recognized. There is no underlying reason that guides or serves as a guarantor. Morality derived from an understanding of people with dignity weighted by the personal is a precarious morality. A person's significance dissolves with the preference or utility of others.

Assisted dying is deemed wrong because it originates from a pseudo morality emerging from a disfigured starting point. It is a starting point that is self-defeating; it undermines the individual, whom it is trying to "liberate" and it does this by reprioritising a person's choice above their worth. This misshapen form of dignity is morally unhealthy—it is intrinsically wrong. Its form allows it to degenerate and it births morally unhealthy products. Assisted dying is but one other.

V. CONCLUSION

The history of ethics has been said to demonstrate the progression of individualism's corrupting influence (Perrau-Saussine, Pinkoski, and Manent, 2022). In this essay, I have argued that assisted dying arises from the moral foundation of dignity whose form is detrimentally altered by individualism's influence.

This moral foundation is deficient and misleading. To be a suitable moral foundation, it would need to ensure that what it unconditionally values—the individual's ability to choose—is genuinely that. That is, there can be no doubt that the most irrevocable of decisions are made with faultless clarity of thought, are impregnable to influence, are made in the stillness of a detached void, and do not result in bad ramifications that affect others. The problem is that none of this is certain or even accurate in the slightest. Rather, we are vulnerable, have cognitive frailties, and are unavoidably embedded in a social network on which we both act and are acted on. Furthermore, when assisted dying is instituted, it brings with it a variety of coercive forces that exert pressure on us as the vulnerable human beings we *actually* are. This deficiency is further magnified by an inherent contradiction that arises. When "choice" is considered moral per se, it contradicts itself when it results in coerced choices that make them wrong.

While these deficiencies reveal a foundation that is morally misleading and inadequate, further criticism condemns it as simply an immoral, self-defeating starting point. Assisted dying arises from

a dignity that has a shape that corrupts itself. In the process of exalting the person's choice, it removes the very basis for doing this and exposes the person to the preferences and utility of others. It is this form of dignity that has provided the substrate for a range of historically oppressive and immoral horrors and that provides the moral starting point for assisted dying.

Assisted dying is grounded on a moral foundation that is not only deficient and self-defeating, but also intrinsically immoral. For these reasons, when assisted dying emerges in a society, it is not morality that prevails, but rather an individualistic form of dignity—*commonly expressed* as "autonomy"—that comes to predominate.

ACKNOWLEDGMENTS

To Mary, John, Brad, Yvonne, Hilary, Daniel, Oliver, and Lucy: thank you for your presence, support, and interest. Thank you to Angus Dawson for your perceptive interaction and guidance during this project's formative stages at the University of Sydney. Thank you to the two independent reviewers of this article who gave it careful scrutiny and provided insightful comments. This is an adaptation of research that was undertaken at the University of Sydney after receiving the University of Sydney Bioethics Scholarship. The views expressed in this work are entirely those of the author and do not necessarily represent any organization with which he is affiliated.

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